



Modified Essay Questions

MARKING GUIDE

MARCH 2026

INSTRUCTIONS:

- Please use pencil ONLY.
- Do not fold or bend.
- Erase mistakes fully.
- Completely fill in the oval.



Please MARK
LIKE THIS ONLY:



Modified Essay 2

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in a regional community mental health clinic. A general practitioner asks you to see Janine, a 39-year-old public school teacher, for an assessment because she has been using excessive amounts of oral morphine obtained from a number of general practitioners. She is married to a builder who is currently unable to work because of a chronic back injury, and they have three children aged 19, 16, and 5 years. She is medically well and not prescribed any other medication.

Janine tells you that she began using oral morphine six years ago after it was prescribed for her husband's back pain. Initially she used it once every few weeks to assist with sleep, but she has been using it every day over the last few months and has noticed that she now requires higher doses. She insists that she only uses it at night to help her sleep, and that she has never used it during the day while working in the school.

Question 2.1

Outline (list and justify) the key aspects that you would want to cover in your initial assessment.

Please note: a list without any justification will not be awarded any marks.

(7/26 marks)

A.	Substance use: • Full drug and alcohol history, in particular assessment of dependence and co-morbid substance use. • Check state or national 'safe script' system, if available, for other prescriptions. • Risks related to substance use – employment, relationship, physical health (e.g. seizures/hepatitis C), her children and those in her care, legal, financial and to self. Reputational risks. • Full history of attempts to cease or reduce use and insight, motivation to abstain and readiness to change. • History of engagement of general practitioners – how many and how often. • Sources of opioids (where and how). • Husband's use of opiates (if relevant). Diversion.	☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
B.	Psychiatric history: • Symptoms of depression or depressive disorder. • Symptoms of anxiety. • Thoughts of harm to self or others. • Sleep history, sleep hygiene, nightmares, stated reasons for poor sleep. • Any associated psychotic symptoms.	☐ 0 ☐ 1 ☐ 2
C.	Functional history, impairments and family issues: • Impairment in work due to harmful substance use including disciplinary procedure. • Impact of her illness on family. • Family stress – caring for three children and her husband. • Relationship with her husband. • Financial issues with husband not working.	☐ 0 ☐ 1 ☐ 2 ☐ 3
D.	CANDIDATE DID NOT ATTEMPT	☐
E.	DID HANDWRITING AFFECT MARKING?	☐

NOTES TO MARKER

- **DID NOT ATTEMPT:** If the candidate did not attempt this question, fill in ONLY the CANDIDATE DID NOT ATTEMPT bubble. No other bubbles should be filled in.
- **MARKS:** This question is worth 7 marks, however, a total of greater than 7 is acceptable.
- **CHECK:** You have marked one bubble for each sub question and initial the box once you have completed marking.



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After you have conducted your initial assessment, you concluded that there is no comorbid psychiatric disorder.

Question 2.2

Describe (list and explain) the management considerations for Janine's substance use.

Please note: a list without any explanation will not be awarded any marks.

(12/26 marks)

A.	Psychoeducation and treatment planning: - Advise and education on diagnosis and prognosis. - Education around potential risks to self, family and career if not managed.	0 1
B.	Work capacity: - Whether needs to stop working while getting treatment. - Workplace safety.	0 1
C.	Pharmacological treatment: Acute withdrawal management - either inpatient or outpatient: - May require symptomatic treatment if required e.g. antispasmodics, antiemetics, antidiarrhoeal agents, simple analgesia and hypnotics. - Aim for gradual morphine dose reduction. - Setting of treatment to be guided by withdrawal symptom severity that can be assessed with an appropriate scale (e.g. COWS). Maintenance treatment: - Long term maintenance treatment with an opiate substitute such as methadone or buprenorphine or other medications e.g. naloxone.	0 1 2 3
D.	Psychological treatments: - Motivational interviewing to develop and improve insight into opioid use disorder. - Alcohol and drug counselling to develop insight and engage in treatment. - Cognitive behavioural therapy - may particularly be useful if Janine has any significant depressive or anxiety symptoms. - Narcotics Anonymous or other support group to enable Janine to engage with others and establish a social support network. - Acceptance and Commitment Therapy.	0 1 2 3
E.	Family support: - Support in parenting role if required particularly around finances if relevant, including access to government financial support if required (for husband and children). - Consider couples' work if relevant to Janine's presentation. - Consider engaging with Janine and husband together to have a shared understanding of current situation.	0 1 2
F.	Sleep hygiene and management: Ensure advice is provided around Sleep Hygiene, such as limiting devices before bed, regular exercise and relaxation techniques, and consider non-addictive medications such as clonidine.	0 1
G.	Professional liaison: - Consider appropriate liaison with educational authority if necessarily. - May require discussion with senior colleague, clinical director or indemnity provider around statutory obligations. - Consider a referral to social work regarding family situation, to ascertain if child protection services may need to be involved. - Consider strategies to minimise multiple prescribers e.g. involving GP's or pharmaceutical body notification; controlled dispensing e.g. for husband if still using opiates; state or national 'safe script' system, if available, for other prescriptions.	0 1 2 3
H.	CANDIDATE DID NOT ATTEMPT	0
I.	DID HANDWRITING AFFECT MARKING?	0

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NOTES TO MARKER

- **DID NOT ATTEMPT:** If the candidate did not attempt this question, fill in ONLY the **CANDIDATE DID NOT ATTEMPT** bubble.
No other bubbles should be filled in.
- **MARKS:** This question is worth 12 marks, however, a total of greater than 12 is acceptable.
- **CHECK:** You have marked one bubble for each sub question and initial the box once you have completed marking.



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After you have conducted your initial assessment, you concluded that there is no comorbid psychiatric disorder.

Janine engaged successfully in outpatient treatment and manages to abstain from further oral morphine use. However, several months later at a follow-up visit, she raises concerns about her youngest child, Marianne. Marianne is not settling well into school and is clingy and oppositional.

Question 2.3

Outline (list and justify) the possible contributors to Marianne's current difficulties.

Please note: a list without any justification will not be awarded any marks.

(7/26 marks)

A.	Attachment issues: • Behaviour problems or anxiety disorder secondary to parental inattention with Janine being busy/pre-occupied. • Lack of responsiveness to Marianne's needs. • Separation Anxiety.	☐ ☐ ☐
B.	Family factors: • Current family stressors including father's chronic illness and mother engaging in therapy and opiate use. • Conflict or problems with siblings, particularly given age gap. • Possible conflict with between Janine and husband adversely affecting Marianne.	☐ ☐ ☐
C.	School factors: • Adjustment to a new environment. • Bullying in school by other peers.	☐ ☐
D.	Parental Factors: • Change in Janine's awareness leading to recognition of long standing problems for her children.	☐ ☐
E.	Child Factors: • Temperament. • Difficulty with developmental transition to school. • Possible communication, intellectual or learning problems, possible intrauterine exposure to Opiates.	☐ ☐ ☐
F.	CANDIDATE DID NOT ATTEMPT	☐
G.	DID HANDWRITING AFFECT MARKING?	☐

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